

BTEC Level 3 Health and Social Care

Unit 2: Working in Health and Social Care

Paper code: 31491H | Session: June 2025 | Total marks: 80

Mark Scheme (Model Answers)

Model answers written to the BTEC Unit 2 command-word structure. These are not the official Pearson mark schemes.

Section A

1 (a) (2)

- Diagnosing Rowan's condition / prescribing treatment / referring to specialists (any two).

1 (b) (4)

- Health and safety policy · protects patients from hazards and accidents.
- Safeguarding / infection control policy · protects patients from harm and infection.

1 (c) (6)

- Maintaining confidentiality and following data protection legislation.
- Storing records securely and sharing only on a need-to-know basis.
- Obtaining consent and keeping records accurate and up to date.

1 (d) (8)

- Person-centred care meets Rowan's individual needs and preferences.
- It promotes Rowan's dignity, choice and independence.
- It leads to better outcomes and greater engagement with care.
- However, it requires time, resources and genuine commitment from staff to be effective.

Section B

2 (a) (2)

- Respecting Kalpesh's dignity / choice / privacy (any two).

2 (b) (4)

- Through supervision and appraisal.
- Through observation and feedback on their work.

2 (c) (6)

- Staying calm and mediating between the young residents.
- Listening to both sides and helping them find a resolution.
- Setting clear boundaries and involving others if the conflict escalates.

2 (d) (8)

- A youth worker provides specialist support and advocacy for Kalpesh.
- They build trust and act as a positive role model.
- They provide access to activities and opportunities for development.
- However, the benefits depend on the quality of the relationship and available resources.

Section C

3 (a) (2)

- Ensuring privacy / showing respect / offering choice (any two).

3 (b) (4)

- Assessing Scott's needs and providing equipment and adaptations.
- Supporting Scott's rehabilitation and independence.

3 (c) (6)

- Sharing information and coordinating care between professionals.
- Using complementary roles to address different aspects of Scott's rehabilitation.
- Working towards shared goals through regular communication.

3 (d) (8)

- CPD keeps professionals' skills and knowledge up to date.
- It improves the quality of care provided to Scott.
- It helps professionals meet registration and revalidation requirements.
- However, CPD requires time and cost, and needs employer support to be effective.

Section D

4 (a) (2)

- Peer support / shared experience / information and advice (any two).

4 (b) (4)

- Assessing the family's needs and developing a care plan.
- Safeguarding the family and advocating on their behalf.

4 (c) (6)

- Treating the family fairly regardless of their characteristics.
- Challenging any discrimination the family may face.
- Providing person-centred, culturally sensitive support.

4 (d) (8)

- Language and communication barriers may prevent access.
- Cultural barriers may make services feel inaccessible or inappropriate.
- Financial and geographical barriers may limit access to services.
- However, these barriers can be reduced through support, advocacy and reasonable adjustments.

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